Trainee Packet

06/29/2020

Purpose of Street Medics:

1. To spread calm
2. To be a resource
3. To prioritize preventative care
4. To know what to do when you don't know what to do
5. To offer medical care when needed

Buddies/PEARL: If possible, it's best to operate in pairs and trios. This is a guideline to share important information with buddies both new and old:

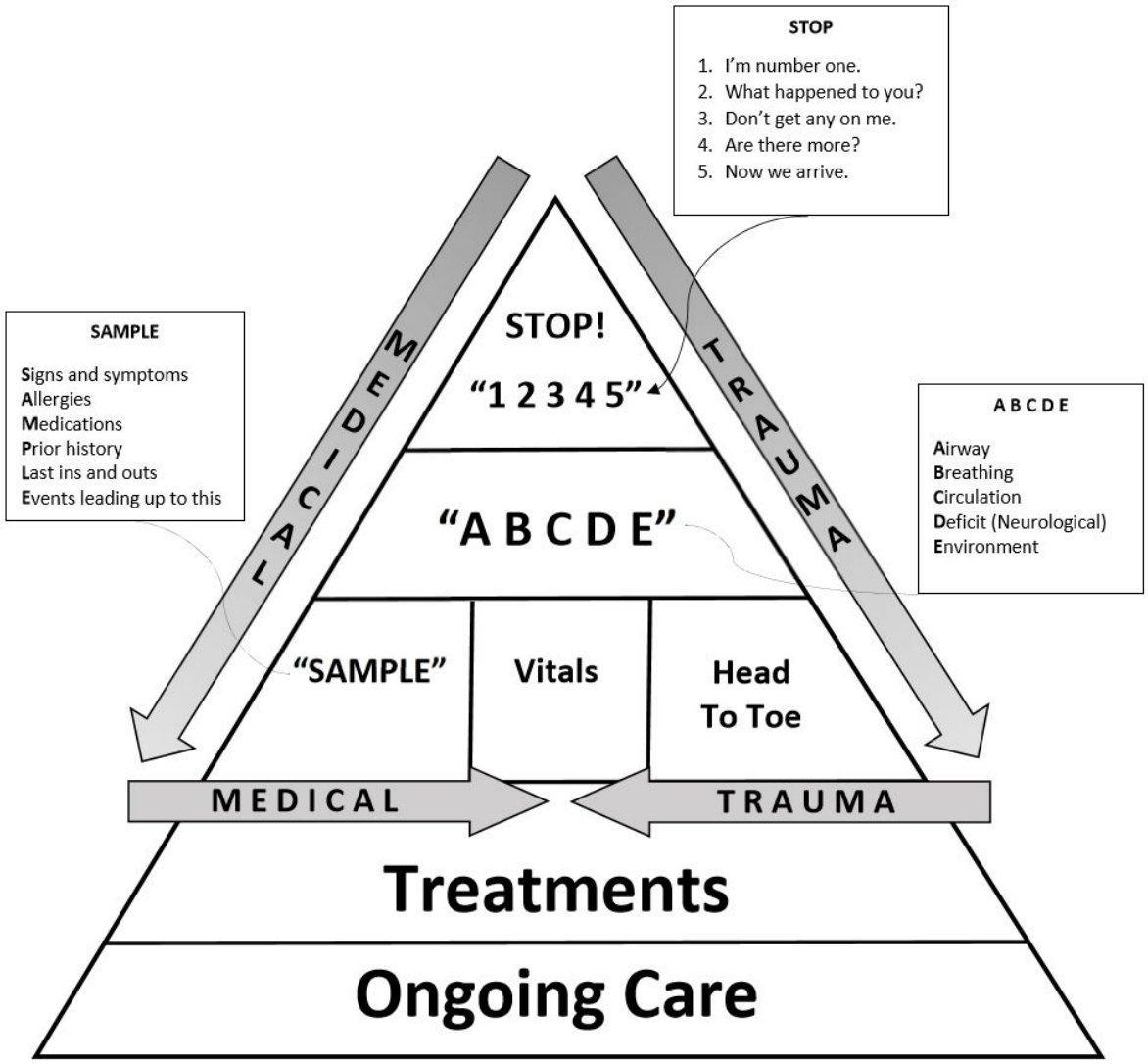
P. Physical vulnerabilities
E. Emotional vulnerabilities
A. Arrestability (Red, Yellow, Green. Go with the lowest)
R. Roles (who takes lead in what situations)
L. Loose Ends (anything else partner should know)

Ethics:

Do no harm! Only perform procedures which you are BOTH trained to do AND confident in doing. Do not abandon a patient unless you transfer care to a person of equal or higher training or the situation becomes unsafe.

Consent:

After introducing yourself to a patient, always get explicit consent to provide care. Do not provide care without this consent, even if you suspect altered mental status. If the patient is unconscious AND unwakeable, you can proceed with IMPLIED CONSENT because a reasonable person would want care if they were unwakeable. If a minor needs and consents to care and you make an unsuccessful, good faith effort to find the parent, administer care. Always respect the wishes of the parent when treating a minor.



Scene Assessment (STOP 12345):

When a dangerous situation or medical emergency occur, you must take 15-30 seconds to assess the scene. If the injured person is going to die in 30 seconds, there wasn't much you could have done in the first place.

STOP: Literally stop don't even walk.
1. I'm #1: Take a careful look not at the patient from a distance, but at the whole scene. what could harm me? Cops? Belligerents? Chemical weapons? Other Hazards?
2. What Happened to you: Trauma? Medical? A Fall? A blow? Cspine precautions?
3. Don't get any on me: Put on gloves. Consider other vectors and types of contamination, squirting blood, tear gas, vomit, other bodily stuff.
4. Are there more: take another look around for other patients. Prioritize.
5. We arrive: Establish roles, make a basic plan (crowd control, for example), remain calm, regulate your breathing, and move in.

Primary Survey (ABCDE):

A	**Assess Airway:** **Look** for loose teeth or other obstructions like vomit or blood. **Listen** for noise such as gargling or snoring. **Feel** for air movement from nose and mouth. **Ask** if there is anything in their mouth.
B	**Assess Breathing:** **Look** for rise and fall of the chest and amount of effort used. **Listen** for breath sounds, ability to speak, gasping. **Feel** for crepitus, chest wall integrity, and movement symmetry. **Ask** if there is any pain or abnormality with breathing.
C	**Assess Circulation:** **Look** at skin color, major bleeding (blood sweep) and capillary refill. **Listen** to their chief complaint and speech as indicator of LOR. **Feel** for presence and quality of pulse. **Ask** if they know of any bleeding.
D	**Assess for (neuro) Deficit:** **Look** at patient behavior and affect, check that pupils are PERRL. **Listen** to how the patient is speaking, whether they are coherent. **Feel** for Cspine integrity, equal control of fingers and toes. **Ask** if they know their name, where they are, what time of day it is, and what happened.
E	**Protect patient from the Environment** Control for hot, cold, wet, wind, and contamination. Remove from extreme cold/heat if at all possible. Shield from elements with anything at your disposal. Talk to your patient and ask how they are feeling. As safely as possible and if necessary, move patient from other threats.

Immediate interventions:

Cspine precautions: . Always hold spine in position found (if possible), do NOT adjust position unless necessary for ABC intervention. Immediately do this and tell patient not to move head in these situations:
1. Confirmed head or neck injury,
2. Abnormal "D" check without a confirmed non-trauma cause,
3. High velocity injury (falls, bike and car accidents, etc),
4. If you for another reason suspect a possible Cspine injury. As long as it doesn't prevent a needed ABC intervention, it doesn't hurt to take Cspine precautions!

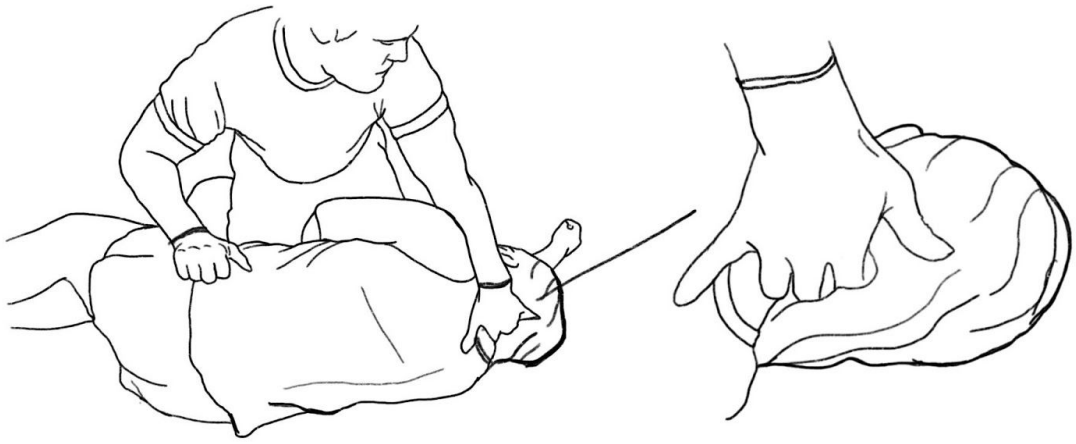
Remember, unless you can't maintain the patient's ABCs otherwise, once you begin to hold cspine, you don't let go until someone can take over or you can improvise a C collar! Also remember that at the end of the day, Cspine precautions are a "D" issue.

A interventions:

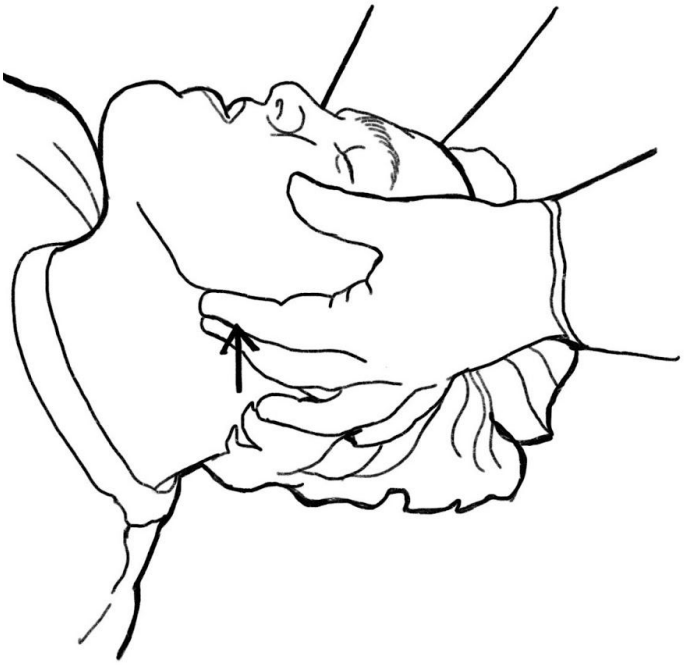
Guttering: If there are obstructions such as gum, vomit, or blood in an unconscious patient's mouth, using a gloved finger, sweep obstructions out along the inside of the cheek, taking care not to put finger between teeth.



	Cspine precautions	**Non-Cspine precautions**
Snoring	Jaw Thrust	Head-Tilt, Chin-Lift
Gurgling	Log Roll	Turn Head









Illustrations by Edna Jones

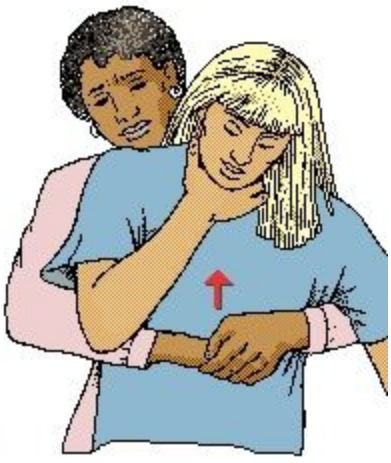
Choking:



Stand or kneel behind the person choking. Place one arm around her waist with the fist positioned between the navel and ribcage. The thumb should be inward directly against her abdomen.



Place the other hand over the first.



Use the outer hand to increase the force as you press inward and upward, quickly forcing air out of the victim's lungs. If this fails to eject the obstruction, repeat the maneuver as many as 4 times.

B interventions:

Inadequate Breathing:

Symptoms:

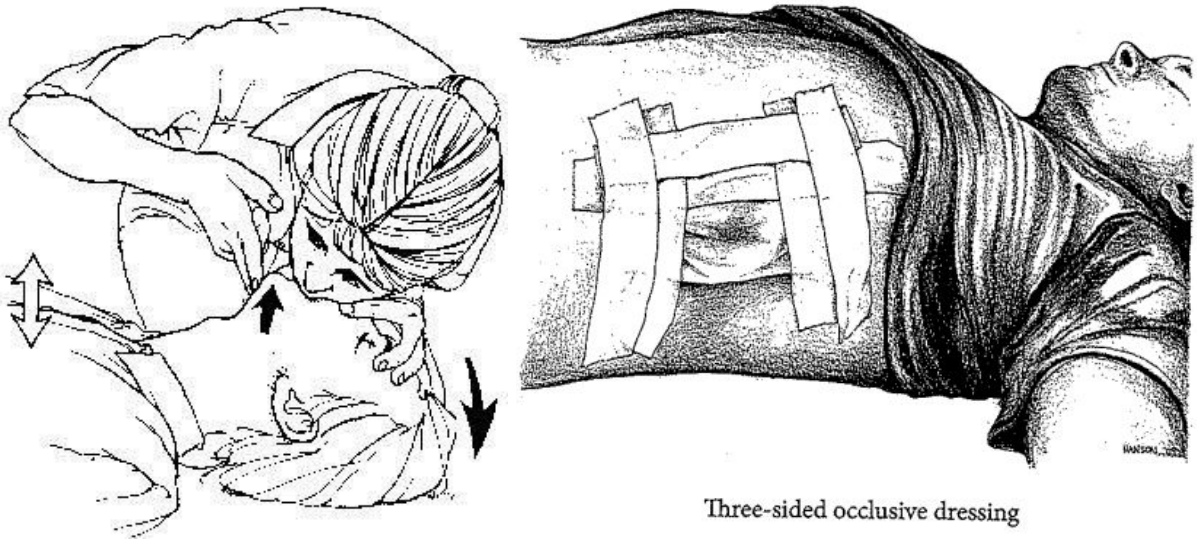
1. Patient not breathing, but does have a pulse; or

2. Patient breathing is shallow and patient appears cyanotic (lips turning blue); or

3. Patient not breathing at least 6 breaths per minute.

Treatment:

Rescue Breathing - Ideally with a CPR mask of some kind: pinch the nose, open the airway (with either head-tilt chin-lift or jaw thrust depending on cspine precautions), make a seal between your mouth and the patient's mouth and give one breath every 5 seconds. Take care not to give too much air; just enough to see chest rise and fall. Remember, patient can be conscious and still require rescue breathing. For example, a patient could get fatigued during an asthma attack and need rescue breathing.



Three-sided occlusive dressing

Puncture Wound (open pneumothorax):

Symptoms:

1. There is a deep wound on the thorax (area from the collarbones down to the lowest rib, both on the front and back of the body).

2. There is evidence of air movement through that wound; either with bubbling or noises.

3. Patient is experiencing ever increasing shortness of breath.

Treatment:

3 sided occlusive dressing - Using a piece of airtight material (a plastic bag, or a rubber glove for example,) create a seal over the wound and tape 3 sides of it, leaving the bottom (relative to gravity) open. Ensure there is a tight seal. If you cannot get a good seal, tape the 4th side down. The 3 sided dressing is meant to act as a one-way valve that allows fluid to escape through the bottom. If you are forced to tape down the 4th side, keep an eye on it and "burp" it when air or fluid accumulates. Keep the patient sitting upright if possible.

Broken Ribs: Will present with point tenderness, possible bruising, and possible pain with deep breaths.

For flail chest (detached rib section, moving opposite the rest of the ribcage), apply a bulky dressing (a bunched up hoodie, for example, over injury) and try to lay patient on wounded side if possible

***For impaled objects, DO NOT REMOVE and apply dressing that will reduce object movement as much as possible.**

C interventions:

Managing Extremity Bleeds: Tourniquet is not necessarily the first technique to try. The only exception to this are massive bleeds as with amputations or major artery bleeds (femoral or brachial) If the wound is deep or large, start by packing it with gauze.
1st: Try direct pressure. It may take a few tries to find the spot that massively reduces bleeding. Once you find the spot, hold a roll gauze on it and see if the roll soaks through or not. If it doesn't, you have likely controlled the bleeding. If that doesn't work, use a pressure wrap.

2nd: A pressure wrap is a tightened ACE bandage over gauze over the wound. A pressure wrap with an ACE bandage should be tight enough to reduce circulation (slow cap refill will indicate this) but not eliminate circulation. If that doesn't work (blood drips through the ACE bandage), use or improvise a tourniquet.

3rd: Using or improvising a tourniquet. Apply tourniquet 2 inches above the wound and tighten as much as possible before using windlass. Tourniquets should be an inch or two above joints, never on a joint. Turn windlass until bleeding stops and then another half turn or so before securing the windlass.

How to make an improvised tourniquet: you need an arms length of sturdy fabric and a thin, sturdy stick, such as a pen or screwdriver. Tie a tight surgeons knot where you would put a tourniquet normally. Place the stick over the surgeons knot and tie a square knot tightly over it. Turn the stick until bleeding stops, a little farther, and then secure the stick.

Don't forget to write the time of tourniquet application on the tourniquet itself or the patient. This is extremely valuable information for the surgeon who will try to save this person('s limb).

Managing Lower Torso Bleeds: Bleed will mostly be internal. Apply pressure with gauze, but do NOT pack it. There is not a whole lot you can do outside the hospital.

Managing Upper Torso Bleeds: Any puncture in the ribcage should be treated as a "B" intervention for open pneumothorax; a 3 sided occlusive dressing. Non-puncture bleeds should receive direct pressure with gauze. No circumferential bandages.

Managing Head and Neck Bleeds: For a neck bleed, apply direct pressure with gauze. For head bleeds, make a donut shaped gauze bandage, place it with the wound in the center, and apply *light* pressure, so as to not push on possible skull fractures. No circumferential bandages.
***For impaled objects, DO NOT REMOVE and apply dressing that will reduce object movement as much as possible.**

Emergency Moves:











Medical History(SAMPLE):

S	**Signs and Symptoms - OPQRST** **Onset -** Did it come on gradually or suddenly? **Provokes or Palliates -** What makes it feel better or worse? **Quality -** What does it feel like? Sharp/dull, stabbing/throbbing, etc. **Radiates** - Is it in one spot or does it extend outward? **Severity -** On a scale of 1-10, 10 being the worst pain you've ever had, what would you rate this? **Time -** What time did this start at?
A	**Allergies** Food, Medications, Insects, Animals, Plants, etc.
M	**Medications** Are you taking any medications? Have you recently stopped taking a regular medication? (with assurance of confidentiality) Are you taking any unprescribed medications(drugs) or recently stopped taking a regular unprescribed medication?
P	**Past Pertinent Medical History** Has this happened to you before? If so, what was the diagnosis? If so, what makes it better or worse?
L	**Last Ins and Outs** When did you last eat and drink? What was it? (if applicable) Has everything been normal with going to the bathroom? If not, ask pointed questions. Menstruation? Possibility of pregnancy?
E	**Events Leading Up To Problem** What *exactly* happened before and during the onset of this problem? (if applicable) Did anyone witness this? What did you do the past 24 hours?

Vital Signs:

1. **Level of Responsiveness (LOR)**

   AVPU Scale:

   Alert and Oriented - The patient is oriented to - 1. Person 2. Place 3. Time 4. Events

   ↓ **A&O X 4:** ▬▬▬

   ↓ **A&O X 3:** ▬▬

   ↓ **A&O X 2:** ▬

   ↓ **A&O X 1:** ▬

   Responsive to Verbal stimulus - Patient only follows commands (e.g. "Blink your eyes")
   Responsive to Painful stimulus - Patient only responds to sternum rub or skin pinch
   Unresponsive - Patient does not respond noticeably to any stimulus

2. **Heart Rate (HR)** - Count the pulse for 15 seconds and multiply by 4

   Rate: 60-100 beats per minute in adults is a normal range
   Rhythm: Regular or Irregular
   Quality: The location taken (radial, brachial, carotid), and its strength (weak, strong, thready, bounding)

3. **Respiratory Rate (RR)** - Count each chest rise for 15 seconds and multiply by 4

   Rate: 12-20 breaths per minute in adults is a normal range
   Rhythm: Regular or Irregular
   Quality: The effort used to breathe - Unlabored or Labored

4. **Skin** - Assess the Skin's Color, Temperature and Moisture (SCTM)

   Color: Look at non-pigmented areas (fingernails, mucous membranes inside mouth, eyelids). Pink (normal), Pale, Cyanotic (bluish)
   Temperature: Hot, Cold, or Warm (normal)
   Moisture: Diaphoretic (sweaty) or Dry (normal)

5. **Pupils** - Check for equality between the right and left pupils and check for reactivity to light

   PERRL: Pupils are equal, Round, and Reactive to Light
   These are normal findings. Note and document any differences.

Time	LOR	RR	HR	SCTM	Pupils
1635	A&Ox4	22 normal	110 strong, regular	P/W/D	PERRL
1640	A&Ox4	20 normal	106 strong, regular	P/W/D	PERRL

Secondary Survey (Head to Toe):

Head/Face

- Look into the **mouth** for blood, vomit, loose teeth or other obstructions.
- Look for blood or cerebrospinal fluid (CSF) draining from **ears** or bruising behind **ears.**
- Look for blood or CSF draining from the **nose.**
- Inspect the scalp for stability of the skull bones, bleeding and bruising by removing clothing and feeling through the hair.
- Evaluate and palpate cautiously for facial fractures.
- Smell the patient's breath for abnormal odors (e.g., alcohol, vomit, etc.).
- Recognize potential airway swelling dangers.

Cervical Spine (C-Spine)

- Look at all parts of the spine for bruising, swelling, or deformity.
- Palpate down the length of the c-spine, feeling each spinous process and checking for point-tenderness, pain, swelling, crepitus, or deformity.

Neck/Throat

- Visually examine the throat for any injuries or deformities.

Clavicles

- Palpate along each clavicle one at a time, feeling for deformity, tenderness, or pain.

Chest

- Look at the chest wall, front and back, for obvious wounds.
- Note breathing effort, pain, crepitus, symmetry of chest wall, and expansion or unusual movement.
- Use both hands to apply resistance to the ribs as the patient takes a deep breath.
- Using the edge of one hand, apply resistance to the sternum and note any pain, crepitus, or instability.

Abdomen (ABD)

- Look at the abdomen for distention (bloating) or bruising.
- Palpate all four quadrants of the abdomen with flat fingers, one quadrant at a time, checking for rebound tenderness, feel for softness, rigidity or abnormal masses. Note any tenderness.

Pelvis

- Press up and in on the iliac crest of the pelvis, then down and out.
- If any instability, crepitus, or pain is noted, **stop** any manipulation immediately.

Legs/Feet

- Look for deformity, discoloration, or obvious injury.
- Palpate the length of each leg for tenderness or deformity.
- After assessing long bones, move hip, knee, and ankle to ensure proper function (or ask patient to)

Arms/Hands

- Look for deformity, discoloration, obvious injury.
- Palpate the length of each arm for tenderness or deformity.
- After assessing long bones, move shoulder, elbow, and wrist to ensure proper function (or ask patient to)

Spine

- Evaluate by palpating each spinous process from the c-spine to the sacrum and log roll as needed.
- Note any pain, tenderness, bruising, swelling, crepitus, deformity, or anything else unusual.

Bleeding and Shock:

Background

Definition: Shock results when brain cells do not receive enough oxygenated blood (tissue and cells lack profusion) and cannot remove wastes, like acids.It is your body's attempt to compensate.

Causes: There are many causes of shock. Most relate to three main types:

- **Hypovolemic Shock:** Problems with the fluid (blood volume)
- **Cardiogenic Shock:** Problems with the pump (cardiac function)
- **Vasogenic Shock:** Problems with the vessels (vascular dilation, anaphylaxis, sepsis, psychogenic and neurogenic)

Details: For this training, the focus is **hypovolemic shock** which can be obvious (external) or subtle (internal). **You must *stop* external bleeding and *assess* for internal bleeding.** S/S of internal bleeding include S/S of shock, bruising, tenderness, distension(severe swelling), or extreme firmness of the abdomen.

Signs & Symptoms of (Hypovolemic) Shock

Shock is progressive. The body gradually loses the ability to compensate for the circulatory deficiency as the patient presents with worsening signs and symptoms, eventually resulting in death.

1. LOR: Anxiety progressing to confusion and unresponsiveness. Sense of doom.
2. HR: Rapid, distal pulses becoming progressively weaker
3. RR: Rapid and shallow (airway constriction in anaphylactic shock)
4. Skin: Pale (possibly cyanotic), cool, clammy, or sweaty (swelling and redness in anaphylactic shock)
5. Pupils: Sluggish

Note: The hallmark of a body no longer able to compensate for circulatory collapse is a drop in blood pressure (BP). Without a BP cuff, this manifests with a loss of (previously confirmed) pulses near extremities and significant drop in the level of responsiveness.

Treatment for Shock

1. ABCDEs - Establish and protect the airway, assist breathing if necessary.
2. Insulate patient from the environment, cover, and keep warm.
3. **Comfort, Calm, and Reassure the Patient** - When caring for your patient, be sincere, compassionate and act confidently. It is important to not offer false hope, but you should reassure the patient that you will do your best to help.
4. Carefully evaluate trends in vital signs.
5. Any patient experiencing shock should be rapidly transported to the hospital.

Head Injuries:

Background

Who: Injuries to the brain are both frequent and serious. Roughly 50% of trauma deaths in the U.S. are due to head injuries, and they rank as the leading cause of death in persons 1-42 years old.
Details: It may be difficult to identify the severity of a head injury based on a patient's initial presentation because it can take time for the injury to manifest.

Signs and Symptoms of Head Injuries/Increasing Intracranial Pressure (ICP)

1. Altered Level of Responsiveness (LOR) is the hallmark of brain injury. Disoriented, Irritable, and Combative. May appear highly anxious.
2. Repetitive questioning and/or inability to recall recent events
3. Nausea, vomiting, projectile vomiting
4. Lethargy
5. Headache
6. Seizures
7. Signs of skull fracture
8. Unequal Pupils
9. Loss of visual acuity, visual disturbances (seeing stars, etc.) or hallucinations
10. Blood or cerebrospinal fluid (CSF) from ears or nose
11. Posturing- Involuntary flexing of muscles in various ways in response to pain
12. Battle's Sign or Raccoon Eyes

Vital Signs (over time)

1. Decreasing heart rate
2. Increasing blood pressure (radial pulses remain strong - "bounding pulse")
3. Changes in respiratory pattern - Irregular breathing

Treatment for Head Injuries

1. ABCDEs.
2. The index of suspicion for a neck injury is high and therefore the c-spine should be immobilized.

Mild Injuries (headache that decreases over time)

1. Monitor hourly for 24 hours for signs of progressing injury.
2. Get patient to a hospital if headache does not improve or vital signs deteriorate (especially LOR)

Severe Injuries (increasing symptoms)

1. Establish and maintain an open airway. Be vigilant of the airway! Vomiting is very common in head injuries. Head injured patients are airway deaths waiting to happen due to the potential for vomiting, the inability to control the airway due to decreased LOR and the propensity for seizures.
2. Immobilize the c-spine. Elevate head 8-10 inches above the feet without flexing the neck (place the patient's entire body on an incline) Monitor vitals closely throughout transport to the hospital.

Hospitalization Criteria for Head Injuries

1. Any head trauma that results in loss of consciousness
2. Any injury with S/S of increasing ICP

Allergic Reactions and Anaphylaxis:

Antigens/Allergen

Anything that can be ingested or otherwise introduced into the body has the potential to be an allergen. A significant number of reactions are called idiopathic, meaning no specific, known allergen is identified. Listed below are the most common known allergens, but numerous others exist.

1. **Medications:** Penicillin and related antibiotics
2. **Foods:** Peanuts, wheat, nuts, milk, shellfish, eggs, and soybeans
3. **Insect stings and bites:** Bees, wasps, and ants
4. **Other:** Latex, pet dander

Signs and Symptoms - Patients may show a combination of the following symptoms:

1. Runny nose
2. Hives, itching, flushed skin
3. Nausea, vomiting, abdominal cramping, diarrhea
4. Facial and airway swelling, wheezing, difficulty swallowing
5. Chest pain
6. Severe respiratory distress
7. Dizziness and loss of responsiveness
8. Low BP, rapid HR
9. Respiratory, vascular, and cardiac failure (shock)

Treatment

1. ABCDEs.
2. Position of comfort (for respiratory difficulty).
3. Take a set of vital signs.
4. If patient is responsive and can control their own airway, consider assisting the person in taking 50mg Benadryl (diphenhydramine) or other antihistamine by mouth.
5. If patient is responsive with signs of airway swelling, circulatory collapse, and/or respiratory distress, and has their own EpiPen consider assisting them with their own administration of epinephrine. Repeated doses may be necessary.
6. When respiratory distress diminishes, consider assisting the patient in taking 50mg Benadryl (diphenhydramine) or other antihistamine, by mouth.
7. Re-take vital signs.

Hospitalization Criteria for Allergic Reactions

1. If an anaphylactic reaction is suspected.
2. Anyone who needed to use an EpiPen for S/S of anaphylaxis.
3. If S/S are not alleviated by an oral antihistamine.

Asthma:

Causes and Triggers of Attacks

1. Allergens: dust, pollen, pollution, smoke, molds
2. Environmental: Cold, altitude
3. Anxiety
4. Exercise/exertion
5. Respiratory infections

Signs and Symptoms

1. Changes in level of responsiveness, particularly anxiety, restlessness, lethargy
2. Difficulty breathing - prolonged expiratory phase, inability to speak in complete sentences
3. Tripod position
4. Wheezing or lower airway noises of constriction, primarily during exhalation
5. Decreased breath sounds (when using stethoscope)
6. Increased HR and RR
7. Skin dusky to cyanotic (depending on degree of difficulty breathing)
8. Chest pain/coughing
9. Decreased ability to talk

Treatment

1. Airway - Maintain patency (open airway).
2. Breathing - If patient tires, you may need to assist through rescue breathing.
3. Medications - Consider assisting with locating/administering rescue inhalers containing albuterol.
4. Pursed Lip Breathing can have a minor palliative effect.

Hospitalization Criteria for Asthma

1. For any prolonged or severe attacks.
2. For increased frequency using inhaler or of attacks.

Diabetic Emergencies:

Diabetic Emergency - Hypoglycemia or Hyperglycemia

You are most equipped to help patients with low blood sugar. These problems can manifest within minutes and require your immediate attention. Patients with high blood sugar have a longer, steady onset and there is less that can be done outside the hospital setting.

HYPOGLYCEMIA	**HYPERGLYCEMIA**
Causes of Low Blood Sugar Emergencies 1. Person changes daily routine (e.g. goes camping, changes diet, increases exercise) 2. Takes normal insulin but has a low or inadequate caloric intake 3. Takes normal insulin, eats normally, and exercises strenuously 4. Takes too much insulin for diet and exercise levels	**Causes of High Blood Sugar Emergencies** 1. Person forgets to take insulin or blood sugar lowering medication. 2. Takes normal insulin, but eats more calories than usual. 3. Takes normal insulin, but exercises much less than usual.
Signs and Symptoms 1. Changes in responsiveness, confusion 2. Headache 3. Thousand-mile stare 4. Anxiety, restlessness, irritability 5. Sweaty, cool skin 6. Weakness (though feeling weak, these patients might fight hard) 7. Can appear drunk 8. Rapid HR and RR 9. Numbness in fingers and toes 10. Seizures 11. Unresponsiveness 12. Hunger	**Signs and Symptoms** 1. Extreme thirst 2. Frequent urination 3. Drowsiness 4. Nausea 5. Dry skin 6. Blurred vision 7. Sweet smelling breath (in extreme cases) 8. Deep, fast breathing (in extreme cases) 9. Hunger
Treatment	**Treatment**

1. ABCDEs 2. Oral sugar if they can maintain their own airway. ● Prepare a slurry of sugar (sweetened Kool-Aid or Gatorade powder mixed rich in water). ● Sweet candies (a soft candy bar), for them to suck on until dissolved. ● Honey, if available works well. ● Follow this with more complex carbohydrates - oatmeal, rice, granola, etc. 3. If the patient has decreased LOR and cannot maintain an open airway, use thick sugar slurry and apply carefully to the gums. Glucose can be absorbed into the blood through oral mucosa, but this can take *many minutes,* so wait patiently while protecting the airway. Use recovery position to drain the airway. **4. <u>Never administer Insulin.</u>**	1. ABCDEs 2. Thorough SAMPLE to determine severity. 3. Have patient test blood glucose if possible

Hospitalization Criteria for Diabetic Emergencies

1. Any loss of consciousness.
2. Decreasing LOR that doesn't improve within 10 minutes of intervention.
3. Confirmed Blood Glucose over 250.

Cold Emergencies:

Mild Hypothermia Symptoms 1. Shivering 2. Confusion 3. Poor motor skills/"the umbles" stumbles, fumbles, mumbles 4. Apathy/Lethargy 5. Combativeness 6. Initial HR and RR increase, followed by steady decline with severity	**Severe Hypothermia Symptoms** 1. Loss of shivering 2. Severely impaired motor skills 3. Decreasing LOR 4. Unresponsive 5. Dilated or fixed pupils 6. Slow HR and RR
Mild Hypothermia Treatments 1. Get patient out of cold and wind 2. Remove wet clothes(none better than wet) 3. Assess for other injuries 4. Feed patient simple sugars followed by carbs 5. Encourage active rewarming exercises	**Severe Hypothermia Treatments** 1. Handle patient as gently as possible! (lots of fluids and chemicals are out of balance in their body, making the heart very delicate) 2. Prevent further heat loss, but do not begin rewarming until definitively secure from elements (indoors) and ideally at a hospital.

Hospitalization Criteria for Cold Emergencies

1. Any severe hypothermic incident (falling through ice, for example)
2. Any symptoms of severe hypothermia

Heat Emergencies:

Heat Exhaustion Symptoms 1. Skin is pale and clammy 2. headache/dizziness 3. Nausea/vomiting 4. Rapid, weak pulse 5. Muscle cramps	**Heat Stroke Symptoms** 1. Skin is red, hot, and dry 2. Throbbing headache 3. Nausea/vomiting 4. Rapid, strong pulse 5. Possible loss of consciousness 6. Seizures 7. Altered LOR/hallucinations
Heat Exhaustion Treatments 1. Relocate to shade and cool 2. Rehydrate patient (not more than ½ liter per hour) 3. Cool patient	**Heat Stroke Treatments** 1. ABCDEs 2. Remove from heat 3. Rapidly cool by any means available 4. DO NOT GIVE FEVER REDUCERS

4. Record regular vitals until patient is back to normal.	5. Rapid Hospitalization

Hospitalization Criteria for Heat Emergencies

1. Heat exhaustion symptoms don't improve after 12 hours
2. Confirmed body temperature over 103 degrees
3. Any symptoms unique to heat stroke

Chemical Weapons:

Use only water (usually from a water bottle with sport cap) in the eyes for chemical exposure, anything else varies from ineffective to harmful. Use soap and water for the skin if you are in a safe place for thorough decontamination and abundant water for rinsing is available.

In a mass casualty incident, be very careful not to be exposed to chemical weapons yourself. You cannot help if you cannot see and are yourself a vector for contamination. Have those in need walk toward you, follow your voice until you are relatively free from smoke/tear gas/cops/chaos.

Basic outline for an eye flush:

- Hi, my name is _____ I'm a medic, can I help you?
-Are you wearing contacts?
-Please don't touch your face. Try to keep your hands in your pockets.
-Can I touch your face and flush your eyes with water?
-*tilt head, firmly hold eye open, flush from nose across eye 2-3 times*
- Tell them to keep blinking until irritation subsides.
-Give aftercare tips

AFTERCARE:

It will continue to burn, but that the eye flush will massively speed up recovery. Blinking repeatedly helps move the process along. Do not touch your face.

Try your best not to contaminate yourself again, or to contaminate your comrades. Be mindful of contaminated clothes touching the inside of cars, furniture or clothes where you go to wash off.

As soon as you reasonably can, put any contaminated clothing and items in a plastic bag. Wash it 2-3 times in cold water with detergent and without any non-contaminated clothes. Use soap and care when decontaminating items by hand.

When washing off, use soap and be intentional about keeping contaminated water from running down uncontaminated parts of the body. Don't just shower like normal. Start by rinsing hair.

Sexual Assault:

In your role as a street medic, you may encounter someone who has been sexually assaulted or raped. In this situation, it is incredibly important to take your cues from the survivor themselves, and not pressure them into making (or not making) any decisions.

It can be helpful to lay out options for the survivor. While it may be overwhelming for them to consider these options, giving them agency and support in making whatever choice is right for them (even if you disagree) can help them in their recovery from this.

Some of these choices are outlined below:

- A survivor can choose to go to the hospital for treatment of any physical injuries and decline a sexual assault examination and rape kit collection. They also should be able to access emergency prophylactic treatment for STDs or pregnancy if applicable. They may be pressured by doctors or nurses to get one done, so if this is their choice, having a support person with them to help ensure their wishes are followed can be very helpful.
- A survivor can choose to have a sexual assault examination within 5 days of the assault (though sooner is better) and have a kit collected. If the survivor chooses this option, suggest they not shower, and if they need to change, recommend putting the clothing they were wearing into a paper bag to take with them to the exam. During this process, the examiner will collect and document the clothing worn, document any injuries (in writing and often with photos), take swabs, comb hair, take scrapings from under the nails, etc. The person performing the exam typically will ask for consent before each step of the process, but having an advocate present can be

extremely beneficial. Many areas have rape crisis centers who can provide advocates during this process if it is not something you feel able to do. During this process the survivor should be able to access prophylactic treatment as with the above option.

    - It may be important to note that having a kit collected does not mean they have to release the kit for testing or have to press charges or have to be part of a trial. A kit can be held in storage for a year in Michigan (this varies in other states) to allow a survivor time to make a decision about releasing the kit. A police officer may come to pick up the kit, but the survivor may refuse to speak to the police.
    - If the survivor chooses to release the kit, the police will likely contact the survivor to make a police report. The survivor can choose not to speak to or cooperate with the police even if they choose to release the kit for testing.
    - It also may be important to share with the survivor that, in Michigan and many other states, a survivor cannot be billed for the cost of the examination. If the survivor has privacy concerns, their insurance can't be billed, or they are charged in error in the state of Michigan, they can contact SAFE Response at (517) 335-7233.

- A survivor can choose not to do any of the options above. If they choose not to take either of these options, suggest they consider following up with a doctor for STD and pregnancy testing if applicable. It can be helpful to be able to provide information for your local rape crisis center and share with the survivor what some of the resources are at that center. Many provide short term counseling, legal and medical advocacy, group counseling and/or support groups, or any number of related services. In your role as street medics, unless the survivor is someone known to you, it's likely that you won't be in a position to provide ongoing support and it's likely you won't know how things went after your time with the survivor has ended. This is why providing resources for support and follow up is critical to responding to a sexual assault even when the survivor chooses not to take any immediate action.

After an assault and any medical treatment (or not), a survivor can choose to file a police report and move forward with pressing charges, a survivor can choose to participate in accountability or restorative justice processes if the person who committed the assault is known to them, or the survivor can choose to do something entirely different or choose to do nothing at all. It's extremely important that we as medics don't let our feelings about the police get in the way of respecting the agency of the survivor.

For more information about reporting an assault, the Rape Abuse Incest National Network (RAINN) website is a useful resource: https://www.rainn.org/reporting-and-criminal-justice-system

It's understandable if you also need to reach out for support after responding to a sexual assault, and there is no shame in that. It can be really important to debrief with your buddy and/or any support people, keeping in mind the need for confidentiality of the survivor. It can also help to engage in self or community care activities.

As street medics, we are also in a unique position of helping to try to change the conversation on consent. Medics always ask before doing anything (aside from in situations of implied consent) and this is something we can take into our organizing and activist work at large. Talking about consent, normalizing asking consent even outside of romantic or sexual contexts, and engaging in other activities furthering primary prevention of assault are all steps that we as medics, organizers, and activists can take to help fight rape culture.

Acronym/buzzword cheat sheet:

LOR - level of responsiveness(same as LOC)
LOC - level of consciousness (same as LOR)
PERRL - Pupils Equal Round and Reactive to Light
AVPU - Alert, Verbal, Pain, Unresponsive
A+O - Alert and Oriented
HR - Heart Rate
RR - Respiratory Rate
SCTM - Skin Color, Temperature, and Moisture
Hx- history
Tx - treatment
S/S - signs and symptoms
MOI - Mechanism of Injury
C/C - chief complaint
BP - blood pressure
Crepitus - The sound and feeling of broken bones grinding
Cyanotic - blue
Thready - weak and rapid
Bounding - (too)strong and slow